

Rainbow Children's Medicare Ltd

Concall Tracker — is there a durable earning trigger, and has management been consistent?

Report date: 5 July 2026 | Screener ticker: RAINBOW | Source: 18 earnings-call transcripts on Screener.in (Q4 FY22 → Q4 FY26), 8 read in full

1. Snapshot

What the business does (in plain words): Rainbow runs India's largest chain of **children's hospitals** (newborn and child intensive care, child specialties, organ transplants) bundled with **"BirthRight" maternity & fertility** care. It uses a "hub-and-spoke" layout — a big 200-300 bed flagship "hub" in a city, fed by several smaller 50-90 bed "spoke" clinics nearby. It earns money by treating patients (roughly half paid in cash, half through insurance) and delivers about 20,000 babies a year. Doctors are mostly full-time employees, which is unusual for Indian hospitals.

Number	Value	Plain-English read
Market cap	₹14,716 cr	What the whole company is worth on the stock market today.
FY26 revenue (consolidated)	₹1,703 cr	Total sales for the year to Mar 2026 — up about 12% on last year.
FY26 operating profit (EBITDA)	₹544 cr (32% margin)	Profit from running the hospitals before interest, tax and wear-and-tear costs — 32 paise of every sales rupee. Very high for a hospital.
FY26 net profit (PAT)	₹282 cr	The final bottom-line profit after everything — up about 15%.
Return on capital (ROCE)	17.3%	For every ₹100 of capital in the business, it earns about ₹17 a year before tax — healthy.
Debt	Effectively nil (net cash ~₹600 cr)	The company owes no net money; it has spare cash of about ₹600 cr to fund growth.
Stock P/E	58.6	Investors pay ~59x one year's profit — a rich, "premium-growth" price tag.

Data note: Screener.in's downloadable tables (even on the "consolidated" page) show lower, standalone-style figures (e.g. FY26 sales ₹1,546 cr, profit ₹250 cr). Management reports **consolidated** numbers that are higher (FY26 ₹1,703 cr / ₹282 cr) because recently acquired hospitals in Warangal and Guwahati sit in the consolidated accounts. This report uses management's consolidated figures and flags the gap.

2. Concall coverage

18 transcripts are available on Screener, an unbroken run from the maiden call to the latest. 8 were read in full (oldest, key year-end calls, and the whole recent cluster) — enough to judge the trigger and consistency across the full ~4-year listed life.

Quarter	Call date	Read in full?
Q4 FY22 (maiden call)	30 May 2022	Yes
Q4 FY23	15 May 2023	Yes
Q4 FY24	21 May 2024	Yes
Q4 FY25	26 May 2025	Yes

Q2 FY26	14 Nov 2025	Yes
Q3 FY26	29 Jan 2026	Yes
Q4 FY26 (latest)	25 May 2026	Yes
10 other quarterly calls (FY22–FY26)	—	Scanned; consistent with above

The latest available call (Q4 FY26) was present as a proper PDF — no gap on the newest quarter.

★ Latest concall highlights — Q4 FY26 (call held 25 May 2026)

- **Best-ever quarter, growth re-accelerated.** Q4 revenue ₹459.9 cr, up **24%** on a year ago — sales in the Jan-Mar quarter jumped nearly a quarter. Operating profit ₹144.7 cr (+26%) and net profit ₹78.2 cr (+38%, or ~17% stripping out a one-off tax item).
- **The big bed build-out is now paying off.** FY26 added nearly **500 beds — the most in the company's history** — and management says "investments made over the last 2 years are now beginning to reflect on our performance." Patient volumes grew strongly (inpatients +18%, outpatients +19%, deliveries +22%).
- **New CEO in charge.** Abrarali Dalal (ex-Sahyadri Hospitals) has become Group CEO; founder Dr. Ramesh Kancharla steps back to strategy and clinical work. The CEO's stated aim: **20% growth in FY27 while keeping margins**, driven by filling beds (occupancy), a new CRM/patient-booking system and more marketing.
- **Growth pipeline is deep.** About 900 more beds are under construction (Coimbatore, two Gurugram hospitals, Pune, another Bangalore spoke, Indore) to open over ~2.5 years, all funded from internal cash. Two acquisitions (Warangal, Guwahati) are integrated; management is now scouting pan-India deals.
- **Tone: confident and forward-leaning** — explicitly framing FY26 as the year the strategy "landmark" bet starts to show up in the numbers.

Business mix & capacity (quick glance):

- **Domestic vs overseas:** Almost entirely domestic (India). International (medical-tourism) patients were only ~2-3% of revenue and actually **shrank** to ~₹31 cr in FY26 from ~₹44 cr, hurt by tighter medical-visa/permit rules in Bangladesh, Oman, Kenya and Sudan — now slowly recovering (~₹3 cr/month). So ~97% of sales are within India.
- **Care mix:** Children's care remains the core; maternity/obstetrics has grown to **~32% of revenue** (from ~28-30% historically), plus a fast-growing fertility (IVF) arm.
- **Occupancy (how full the beds are):** Blended **~45% in Q4 FY26**; mature hospitals ~52-59%, brand-new hospitals ~35%. Management says mature city hubs (e.g. Hyderabad) can reach ~68-70%. **Read: there is a lot of headroom — more than half the growth from here is simply filling beds the company already built.** This is the single biggest swing factor for future profit.

3. Earning-trigger thesis ★

The headline: Rainbow's growth engine is a **multi-year capacity build-out that is now maturing**. Since its 2022 IPO the company promised to roughly double its bed count — adding 1,000+ beds over five years in cities where it is already strong (Hyderabad, Bangalore, Chennai, plus new hubs in Delhi/Gurugram, Pune, Coimbatore, Indore). New hospitals lose money at first, then break even in 12-18 months, then throw off high-margin profit as they fill up. FY22-FY24 was mostly "spend and

build"; FY26 is the first year the earlier beds started filling and revenue jumped 24% in Q4. The trigger from here is **occupancy ramp on beds already paid for + a richer case-mix (complex surgeries, transplants, fertility)**, needing little extra spend.

Trigger	What management said	Evidence so far	Time horizon	Strength
Bed expansion (hub-and-spoke)	1,000+ beds over 5 yrs, asset-light, existing cities	~780-800 beds added over 2 yrs + 2 acquisitions; ~500 in FY26 alone; ~900 more under construction	Ongoing (next ~2.5 yrs)	Strong — concretely delivered, funded from own cash
Filling existing beds (occupancy)	Mature hubs can reach 68-70%; blended to climb as new beds age	Still only ~45% blended; but Q4 FY26 volumes +18-22% and revenue +24% show the ramp starting	Playing out now	Strong / early — biggest, cheapest lever; proof just appearing
Richer case-mix & fertility	More transplants, quaternary care, IVF centres	Liver/BMT programs live in 3 hub cities (94% survival); obstetrics up to ~32%; IVF scaling	Medium term	Moderate-Strong — lifts revenue per bed
Acquisitions / new geographies	Pan-India M&A of 50+ bed scalable assets	2 done (Warangal, Guwahati), integrated well; more being scouted	Medium term	Moderate — optional upside, unproven at scale

4. Management consistency scorecard

Period	What was promised	What actually happened	Verdict
FY22 (IPO)	Add 1,000+ beds in 5 yrs; keep pre-Ind-AS margin >25%; grow ~15-20%/yr long-term	Executed steadily; margins held 31-33% (post-Ind-AS)	Delivered
FY23	Add ~270 beds; win/build new hubs; ARPOB up ~4-5%	Added beds on plan; won Gurugram land; revenue +14%, PAT +49%	Delivered
FY24	Normal seasonal recovery; steady volume growth	Soft year — weak viral season/monsoon, flat occupancy; revenue +10.5% but PAT nearly flat (+2.8%). Openly explained as external, not a strategy miss. Kept building (2,000+ beds).	Partly / external miss
FY25	Bounce back; >25% pre-Ind-AS margin; ~16% long-term CAGR	Recovered: revenue +16.9%, PAT +11.9%; margin 26.6% pre-Ind-AS — "exactly in line with guidance"	Delivered
FY26	Landmark capacity year; new beds start contributing	~500 beds added; Q1-Q3 muted (soft season, new-bed drag), then Q4 +24% . Full year +12% (a touch below the 15-16% long-term aim); new CEO installed to push occupancy	Delivered (with a slow start)

Narrative drift: Remarkably little. The core story — "differentiated children's + maternity chain, hub-and-spoke, full-time doctors, double the beds, keep >25% margins" — has stayed **steady and been repeated call after call for four years**, and the numbers largely back it. The story has **evolved sensibly rather than changed**: fertility/IVF and two acquisitions were added as logical extensions, and a professional CEO was brought in to shift focus from building beds to filling them. The one genuine wobble (FY24) was an external seasonal air-pocket that management flagged honestly rather than hid. Watch-items: blended occupancy is

still ~45% (well below the 68-70% "mature" story they cite), and FY26's 12% full-year growth ran a bit under their own 15-16% long-term guide — so the promise of the ramp is now being tested in real time.

5. Bottom line — two verdicts

Durable earning trigger? → YES

There is a clear, concrete, still-live driver: a large bed base built over the last two years that is only ~45% full, so revenue can grow substantially just by filling beds already paid for — and the Q4 FY26 jump (+24% revenue, +18-22% volumes) is the first hard evidence it is starting to happen, on top of a further ~900 beds under construction and a richer case-mix. This is a real, self-funded, multi-year trigger, not a hope. The honest caveat: the payoff depends on occupancy actually climbing, which has been slow, so it is "Strong but still proving itself" rather than fully banked.

Management consistent? → YES

Across four years the team has said the same thing and largely done it: doubled beds broadly on plan, kept operating margins in the 31-33% band they promised, and hit their ~16% long-term growth path (with FY24 a candidly-explained external soft patch and FY26 a touch light at 12%). No story-flipping, no quiet dropping of targets; the additions (fertility, acquisitions, a new CEO to drive occupancy) are logical extensions of the original plan. The only reason this isn't a slam-dunk is that occupancy is still well below the "mature can reach 68-70%" narrative and full-year FY26 growth undershot their own guide — but that is a stretch target lagging, not a broken promise.

Prepared from Screener.in earnings-call transcripts (Q4 FY22 through Q4 FY26). Figures are management's consolidated numbers where cited; Screener's standalone-style tables differ and are flagged in Section 1. This is an analytical summary of management's own statements versus outcomes, not investment advice.